

InsureMind AI Medical Center

Prior Authorization Report | Powered by InsureMind AI

PRIOR AUTHORIZATION REPORT

Case ID: HOSP5

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LIKELY APPROVED — 77.8%

PATIENT DETAILS & CLINICAL PROFILE

Patient Name	Uma
Age	70 years
Gender	Female
TPA / Insurer	Demo TPA
Disease Desc.	Patient presents with severe chest pain, hypertension, and shortness of breath for 3 days.
Raw Procedure	surgery

DIAGNOSIS & PROCEDURE

ICD-10 Code	I21.01
ICD-10 Description	ST elevation MI involving left main coronary artery
Procedure Code	surgery
Procedure Description	surgery
Condition Type	Critical
Medications	Aspirin 81mg, Lisinopril 10mg

SOAP NOTE

S — Subjective

Patient reports severe chest pain, hypertension, and shortness of breath for 3 days

O — Objective

70yo female, taking Aspirin 81mg and Lisinopril 10mg, with Demo TPA

A — Assessment

Acute coronary syndrome, consider myocardial infarction or pulmonary embolism

P — Plan

Order ECG, troponin, and chest X-ray, continue Aspirin and Lisinopril, and refer to cardiologist

CLINICAL EVIDENCE

Evidence Completeness	100% complete
✓ Duration of Symptoms	1 year
✓ Prior Treatment	yes
✓ Severity	high
✓ Investigations	ECG done, shows ST elevation
✓ Specialist Referral	referred to cardiologist

CLINICAL JUSTIFICATION

Justification Score	0.92 / 1.00
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The patient, Uma, a 70-year-old female, presents with severe chest pain, hypertension, and shortness of breath for 3 days, indicative of acute coronary syndrome. The electrocardiogram (ECG) reveals ST elevation, a hallmark of myocardial infarction, specifically involving the left main coronary artery (I21.01). Given the high severity of her condition and the duration of symptoms, immediate surgical intervention is medically necessary to prevent further cardiac damage and potentially fatal outcomes. The patient's current medications, Aspirin 81mg and Lisinopril 10mg, are standard treatments for myocardial infarction but are insufficient as standalone therapies in this acute setting. The referral to a cardiologist further underscores the complexity of her condition, requiring specialized care. Prior treatment and the patient's symptoms over the past year highlight the chronic nature of her condition, which has now acutely decompensated. Therefore, surgical intervention is not only justified but required to address the immediate threat to her health and to prevent long-term cardiac sequelae. The clinical evidence, including the ECG findings and the patient's symptomatology, supports the urgency and necessity of the proposed surgical procedure.

RISK FLAGS		
Severity	Flag	Description
MEDIUM	Elderly patient (≥65 years)	Elderly patients carry increased surgical/procedural risk. Enhanced documentation and specialist sign-off recommended.
HIGH	High-acuity diagnosis	ICD-10 I21.01 indicates a serious/complex medical condition. Comprehensive supporting documentation is critical for authorization.

DECISION REASONING & RECOMMENDATIONS

- Approval Decision Reasons:
- Strong clinical evidence provided (100% complete).
 - Clinical justification meets documentation quality standards.
 - Critical condition — strong medical necessity supports authorization.
 - High-severity risk flags detected: High-acuity diagnosis.